BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety	Full blood count, kidney and liver tests; medicine review and pregnancy-prevention programme assessment for women and men. If pregnancy is possible, a supervised pregnancy test is needed within seven days before starting.	Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.
Cancer, skin and daily function	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up covers the treated area and the rest of your skin. The team chooses examinations and scans for your situation. Vismodegib pregnancy-prevention and donation restrictions continue for the periods listed in your safety plan after the final dose.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Vismodegib can cause severe harm to an unborn baby. Women who could become pregnant need the formal pregnancy-prevention programme, including two recommended contraceptive methods (one highly effective method plus a barrier method), unless the agreed alternative is continuous abstinence, during treatment and for 24 months after stopping. Pregnancy tests are supervised before treatment and monthly, even with absent periods. Men must use a condom, with spermicide if available, during sex with a female partner during treatment and for two months afterwards, including after a vasectomy. Report a suspected patient or partner pregnancy immediately. Do not breastfeed or donate blood during treatment or for 24 months afterwards; do not donate semen during treatment or for two months afterwards. Ask the team to explain and document the full programme; this summary does not replace its materials.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00236 v3: current regimen source · EMA Erivedge product information, updated 5 November 2025

THE WHOLE TREATMENT JOURNEY

Vismodegib for advanced basal cell skin cancer: treatment and checks

NCCP 00236: symptomatic metastatic basal cell carcinoma, or locally advanced basal cell carcinoma unsuitable for surgery or radiotherapy.

Take vismodegib once every day at the same time, continuously while the team considers it beneficial and tolerable. A 28-day calendar organises the four-week review; it is not a planned seven-day break or a fixed number of courses.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00236	Full blood count, kidney and liver tests; medicine review and pregnancy-prevention programme assessment for women and men. If pregnancy is possible, a supervised pregnancy test is needed within seven days before starting.	Full blood count, kidney and liver tests every four weeks. Review cramps, strength, taste, eating, weight, hair/skin changes and treatment tolerance. Pregnancy testing is monthly when applicable, even if periods stop.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Agree continuing clinical/skin review, any response imaging and support for ongoing effects. The pregnancy-prevention, donation and breastfeeding restrictions continue after the last dose for their specified periods.
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What each check is for

1 Continuous oral treatment

Daily medicines

Vismodegib is used to help control advanced basal cell skin cancer when the source-specific criteria are met. Discuss whether the aim is shrinkage, symptom relief, longer disease control or another agreed goal, and how surgery or radiotherapy has been considered.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00236

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40